

Overview

- VITL is a Unity-based medical simulation product surface on the MetaDyn platform.
- The current scenario centers on a pediatric infant UTI case where a learner interacts with the infant's concerned mother by text or voice.
- The current near-term goal is Milestone 1: proving a coherent end-to-end learner loop rather than over-claiming full clinical assessment.

Current Product Shape

- AI delivery layer: MetaDynVoiceController handles learner input, transcription, LLM response flow, text-to-speech, and hidden state.
- Simulation-state layer: VITLSimulationManager owns lifecycle state, activity progression, timers, inactivity escalation, learner-timed events, and activity completion.
- Assessment/intake layer: model-backed communication scoring plus a planned structured patient-history intake flow for Activity 2.

Milestone 1 Proof

- Mother opens with concern.
- Learner responds by text or voice.
- Learner communication is scored.
- Simulation state updates before the mother response.
- Activity 1 completes through authored thresholds when appropriate.
- Activity 2 begins as structured patient-history capture.

Key Decisions

- The simulation manager remains the authority for activity progression.
- Conversation history is not the same thing as structured patient history.
- The learner-entered intake UI should become the canonical structured history record for Activity 2.
- Fallback scoring is a resilience mechanism, not the intended long-term assessment layer.
- Milestone 1 should stay focused and avoid scope creep into full clinical grading, automatic structured extraction from dialogue, or full clinical assessment.

Immediate Next Steps

- Lock the canonical Activity 1 to Activity 2 demo path.
- Define and implement the first pass of VITLPatientIntakeRecord and VITLPatientIntakeController.
- Add required-fields plus learner-confirmation completion behavior for Activity 2.
- Add analytics-safe milestone events without over-expanding scope.